

Axcel[®] CHLOROQUINE TABLET

COMPOSITION:

Each tablet contains
Chloroquine Phosphate 250mg
equivalent to Chloroquine base 155mg

PRESENTATION:

A white, flat, round tablet, 11mm in diameter with a single score-line on one side and marked **KOTRA** logo on the other side.

INDICATIONS:

- Treatment and suppression of vivax malaria
- Treatment of *falciparum malariae*
- Giardiasis
- Chronic lupus erythematosus and other skin diseases
- Extraintestinal amoebiasis (amoebic hepatitis) and liver abscess
- Chronic rheumatoid arthritis

PHARMACOLOGY:

Chloroquine is highly active against erythrocytic forms *Plasmodium vivax* and *Plasmodium Malariae* and most strains of *Plasmodium falciparum* (but not the gametocytes of *Plasmodium falciparum*). It is highly effective as a suppressive agent in patients with vivax or malariae malaria, in terminating acute attacks, and significantly lengthening the interval between treatment and relapse. In patients with *falciparum malariae* it abolishes the acute attack and effects complete cure of the infection, unless due to a resistant strain of *Plasmodium falciparum*. *In vitro* studies with trophozoites of *Entamoeba histolytica* have demonstrated that Chloroquine also possesses amebicidal activity comparable to that of emetine.

DOSAGE AND ADMINISTRATION:

1) Malaria (therapy): To be taken before or better after meal. In case of acute attack of malaria, the total dose required usually need not exceed 1.5g of Chloroquine base. The initial dose of 4 tablets is followed by 2 tablets after 6-8 hours and by 2 tablets on each of the two following days. In patients partially immune to the preparation, a single dose of 3 to 4 tablets is usually sufficient to suppress an attack.

2) Malaria (prophylaxis): The following dose is taken once weekly, always on the same time. For prevention of malaria, a dose of 2 tablets is administered during the existence of the risk of infection and for 4-8 weeks afterwards. In

non-immune persons, in area with great risk of infection, a dose of 4 tablets weekly may be required.

3) Treatment of extraintestinal amoebiasis and liver abscess: 1-2 tablets daily for at least 2 or 3 weeks. Children are administered $\frac{1}{3}$ - $\frac{2}{3}$ of the adult dose, according to age and development. In case of a life-threatening liver abscess, the dose on the 1st and 2nd day may be doubled or an injection of chloroquine phosphate solution may be administered.

4) Treatment of lupus erythematosus: 1 tablet once or twice daily for 1 or 2 weeks, followed by a maintenance dose of 1 tablet daily.

5) Treatment of chronic rheumatoid arthritis: 1 tablet once daily after evening meal through several months. In case of adverse reactions, administration should be withheld for several days until they disappear. The treatment should be continued with $\frac{1}{2}$ tablet daily and if well tolerated, increased to 1 tablet. If objective improvement is not observed after 6 months, the drug should be discontinued. In infants and children the dosage is preferably calculated on the body weight.

CONTRAINDICATION:

In the presence of severe gastrointestinal, neurological and blood disorders, hepatic impairment, chloroquine phosphate is to be used with great caution or not at all, depending on degree of injury. Concurrent application of gold or phenylbutazone and chloroquine phosphate is to be avoided.

PRECAUTION:

In patients on long-term treatment, regular examination of eventual ocular disorders should be carried out. It should be administered with great caution in patients with renal or hepatic impairment, with porphyria or psoriasis. Reactions are more frequent in alcoholics. Temporarily, chloroquine may effect visual accommodation and patients should avoid driving or operating machinery until the effects subside. An overdosage may provoke fatal effects, mainly in children. The use of chloroquine is contraindicated in pregnancy unless the risk of malaria is greater than the possible risk of damage to the foetus.

SIDE EFFECTS:

Chloroquine phosphate is relatively well tolerated. Mild and transient headache, visual disturbances, gastrointestinal disorders, and pruritus may be provoked by the quantity of the drug applied in acute malaria attack in prolonged chronic treatment. All symptoms occur in a mild form and rapidly disappear on discontinuation of the drug. The long-term treatment of diseases other than malaria with high doses may cause retinopathy. Visual failure is not necessarily progressive if the drug is discontinued, yet it appears to be irreversible.

OVERDOSAGE AND TREATMENT:

Toxic symptoms may occur within 30 minutes. Treatment is symptomatic and must be prompt with immediate evacuation of the stomach by emesis (at home, before transportation to the hospital) or gastric lavage until the stomach is completely emptied. Finely-powdered, activated charcoal in a dose not less than 5 times the estimated dose of chloroquine ingestion of the antimalarial, may inhibit further intestinal absorption of

the drug. Convulsions, if present, should be controlled before attempting gastric lavage. If due to cerebral stimulation cautious administration of an ultrashort acting barbiturate may be tried but, if due to anorexia it should be corrected by oxygen administration, artificial respiration or, in shock with hypotension, by vasopressor therapy. Because of the importance of supporting respiration, tracheal intubation or tracheostomy, followed by gastric lavage, may also be necessary. Peritoneal dialysis and exchange transfusions have also been suggested to reduce the level of the drug in the blood. A patient who survives the acute phase and is asymptomatic should be closely observed for at least 6 hours. Fluids may be forced, and sufficient ammonium chloride (8g daily in divided doses for adults) may be administered for a few days to acidify the urine to help promote urinary excretion.

STORAGE:

Keep container well closed. Store below 30°C. Protect from light.

KEEP OUT OF REACH OF CHILDREN JAUHI DARI KANAK-KANAK

PACK QUANTITIES:

Available in blister pack of 10 x 10's and plastic container of 1000's (for export / tender only).

Further information can be obtained from pharmacist, physician or the manufacturer.

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